

ED Provider Notes

Signed Jun 19, 2026

ED Provider Notes by William Jones at 06/19/26 2149

CHIEF COMPLAINT

Chief Complaint

Patient presents with

- Failure To Thrive

HPI

Lisa A English is a 61 y.o. female who presents with inability to care for herself at home. She was discharged yesterday for severe COPD with exacerbation. She uses an electric scooter to get around and in the past friends and family been able to help her at home transition between her recliner and the scooter. She thought her son was going to be able to help care for her at home but found out upon arriving home that he has left and will not come back. Home health which plan on seeing her once a week visit her today for the 1st time and actually called the primary and said, via review of the chart, that they did not think that this was going to work out. An ambulance was called when patient came here because she states she can not care for herself and she has no one to care for her at home. She has been in her recliner since being transported back home yesterday.

REVIEW OF SYSTEMS

Twelve point review of systems obtained and otherwise unremarkable except as above.

PAST MEDICAL HISTORY

Past Medical History:

Diagnosis	Date
• Anxiety	07/20/2023
• Arthritis	
• Bilateral ovarian cysts	
• Cataract	04/12/2019
• Chicken pox	
• COPD (chronic obstructive pulmonary disease) (HCC)	
• Coronary artery disease	
• Depression	
• Emphysema of lung (HCC)	04/04/2021
• GERD (gastroesophageal reflux disease)	
• Heart palpitations	
• HPV (human papilloma virus) anogenital infection	02/04/2021

2/4/2021 PS, LPN

• HSV (herpes simplex virus) anogenital infection	
• Hyperlipidemia	03/07/2016
• Hypertension	
• Kidney stone	
• Normocytic anemia	04/17/2026
• PTSD (post-traumatic stress disorder)	
• Tobacco abuse	11/06/2015
• Tubal pregnancy	1988
• Type 2 diabetes mellitus (HCC)	05/05/2026

FAMILY HISTORY

Family History

Problem	Relation	Name	Age of Onset
• Diabetes	Mother	Mary Catt	
• Depression	Mother	Mary Catt	
• High Blood Pressure	Mother	Mary Catt	
• Hypertension	Mother	Mary Catt	
• Alcohol Abuse	Father	Claude Dobbs	
• Diabetes	Father	Claude Dobbs	
• Bipolar Disorder	Sister		
• MI before 55	Brother	Tony Hurst	
• Early Death	Brother	Tony Hurst	
<i>Myocardial infarction</i>			
• Diabetes	Maternal Uncle	Charles hurst	
• Cancer	Maternal	?	
	Grandfather		
• No Known Problems	Son		
• Early Death	Sister	Molly Echols	
<i>COPD</i>			
• Diabetes	Sister	Anita hurst	
• No Known Problems	Son		
• No Known Problems	Son		
• Heart Disease	Neg Hx		

SOCIAL HISTORY

Social History

Socioeconomic History

- Marital status: Divorced
- Number of children: 2

Occupational History

Employer: RIVER CITY TAXI

Tobacco Use

- Smoking status: Former
 - Current packs/day: 0.00
 - Average packs/day: 0.5 packs/day for 45.6 years (22.8 ttl pk-yrs)
 - Types: Cigarettes
 - Start date: 7/7/1978
 - Quit date: 02/2024
 - Years since quitting: 2.3
- Smokeless tobacco: Never
- Tobacco comments:
 - 5/29/24--Smoked some in February 12, 24 but has stopped since then.--11/10/25 Pt has quit smoking.*

Vaping Use

- Vaping status: Former

Substance and Sexual Activity

- Alcohol use: Not Currently
 - Alcohol/week: 1.0 standard drink of alcohol
 - Types: 1 Glasses of wine per week
 - Comment: in AA*
- Drug use: Not Currently
 - Types: Cocaine
- Sexual activity: Not Currently
 - Partners: Male

Other Topics Concern

- Caffeine Concern No
 - Comment: three glasses of tea daily*

Social Drivers of Health

Tobacco Use: Medium Risk (6/19/2026)

Patient History

- Smoking Tobacco Use: Former
- Smokeless Tobacco Use: Never

Alcohol Use: Not At Risk (5/29/2024)

Alcohol Use

- Alcohol Use Status: Not Currently
- Average alcohol consumption: .6

Food Insecurity: No Food Insecurity (6/10/2026)

Hunger Vital Sign

- Worried About Running Out of Food in the Last Year: Never true
- Ran Out of Food in the Last Year: Never true

Recent Concern: Food Insecurity - Food Insecurity Present (3/30/2026)

Hunger Vital Sign

- Worried About Running Out of Food in the Last Year: Sometimes true
- Ran Out of Food in the Last Year: Never true

Transportation Needs: No Transportation Needs (6/10/2026)

PRAPARE - Transportation

- Lack of Transportation (Medical): No
- Lack of Transportation (Non-Medical): No

Intimate Partner Violence: Not At Risk (6/10/2026)

Humiliation, Afraid, Rape, and Kick questionnaire

- Fear of Current or Ex-Partner: No
- Emotionally Abused: No
- Physically Abused: No
- Sexually Abused: No

Depression: Not At Risk (6/18/2026)

PHQ-2

- PHQ-2 Score: 0

Housing Stability: Low Risk (6/10/2026)

Housing Stability Vital Sign

- Unable to Pay for Housing in the Last Year: No
- Number of Times Moved in the Last Year: 1
- Homeless in the Last Year: No

Utilities: Not At Risk (6/10/2026)

AHC Utilities

- Threatened with loss of utilities: No

SURGICAL HISTORY

Past Surgical History:

Procedure	Laterality	Date
• Appendectomy		1978
• Colonoscopy		10/03/2022
• Colonoscopy		
• Ovarian cyst drainage	Left	1978
• Ovary removal		1988
• Port placement	Right	05/06/2022
• Renal artery stent placement		11/2019 ?
• Sinus surgery		2015
• Tonsillectomy		1971
• Tubal ligation		1988
• Ureter stent placement	Right	06/14/2020

Procedure: CYSTOSCOPY, WITH RETROGRADE PYELOGRAM AND URETERAL STENT INSERTION; Surgeon: William C Collyer, MD; Location: MIDTOWN OR; Service: Urology; Laterality: Right;

CURRENT MEDICATIONS

No current facility-administered medications for this encounter.

Current Outpatient Medications:

- acetaminophen (TYLENOL) 500 MG tablet, Take 1 tablet (500 mg) by mouth every 6 (six) hours if needed for Pain, Disp: , Rfl:

- albuterol 108 (90 Base) MCG/ACT inhaler, Inhale 2 puffs by mouth every 6 (six) hours if needed for Wheezing or Shortness of Breath, Disp: 18 g, Rfl: 11
- Albuterol Sulfate (2.5 MG/3ML) 0.083% Inhalation Nebulization Solution, Take 1 vial (2.5 mg) by nebulization every 6 (six) hours if needed for Wheezing or Shortness of Breath Indications: Reversible Disease of Blockage in Breathing Passages, Spasm of Lung Air Passages, Disp: 150 mL, Rfl: 11
- Alcohol Swabs 70 % PADS, Indications: Diabetes Provide per pt preference or as covered by insurance. For use to monitor blood glucose levels as ordered per Provider. DXE11.9 (Patient taking differently: Apply 1 each topically daily Indications: Diabetes Indications: Diabetes Provide per pt preference or as covered by insurance. For use to monitor blood glucose levels as ordered per Provider. DXE11.9), Disp: 100 each, Rfl: 11
- amoxicillin-clavulanate (AUGMENTIN) 875-125 MG per tablet, Take 1 tablet by mouth 2 times daily for 3 days, Disp: 6 tablet, Rfl: 0
- aspirin 81 MG chewable tablet, Take 2 tablets (162 mg) by mouth daily, Disp: 60 tablet, Rfl: 11
- atorvaSTATin (LIPITOR) 20 MG tablet, Take 1 tablet (20 mg) by mouth daily, Disp: 90 tablet, Rfl: 1
- Blood Glucose Monitoring Suppl w/Device KIT, Indications: Diabetes Provide per pt preference or as covered by insurance Check blood glucose levels once a day as ordered per Provider. DXE11.9, Disp: 1 kit, Rfl: 0
- budesonide (PULMICORT) 0.5 MG/2ML nebulizer suspension, Take 2 mL (500 mcg) by nebulization 2 times daily, Disp: 120 mL, Rfl: 11
- busPIRone (BUSPAR) 5 MG tablet, Take 1 tablet (5 mg) by mouth 2 times daily, Disp: 60 tablet, Rfl: 5
- docusate 100 MG CAPS, Take 100 mg by mouth 2 times daily, Disp: 30 capsule, Rfl: 2
- dupilumab (DUPIXENT) 300 MG/2ML pen, Inject 2 mL (300 mg) into the skin every 14 days, Disp: 4 mL, Rfl: 5
- ferrous sulfate 325 (65 Fe) MG tablet, Take 1 tablet (325 mg) by mouth daily (with breakfast), Disp: 30 tablet, Rfl: 2
- Fingerstix Lancets MISC, Indications: Diabetes Provide per pt preference or as covered by insurance Check blood glucose levels once a day as ordered per Provider. DXE11.9 (Patient taking differently: 1 each by Other route daily Indications: Diabetes Indications: Diabetes Provide per pt preference or as covered by insurance Check blood glucose levels once a day as ordered per Provider. DXE11.9), Disp: 100 each, Rfl: 11
- FLUoxetine (PROZAC) 20 MG capsule, Take 1 capsule (20 mg) by mouth daily, Disp: 30 capsule, Rfl: 2
- furosemide (LASIX) 20 MG tablet, Take 1 tablet (20 mg) by mouth daily, Disp: 2 tablet, Rfl: 0
- glucose blood VI test strip, Please check blood sugars once a day per orders (Patient taking differently: 1 strip by Other route daily Please check blood sugars once a day per orders), Disp: 100 each, Rfl: 11
- guaifENesin (MUCINEX) 600 MG SR tablet, Take 2 tablets (1,200 mg) by mouth every 4 hours as needed for Congestion, Disp: , Rfl:
- ipratropium-albuterol (DUO-NEB) 0.5-2.5 (3) MG/3ML nebulizer solution, Take 3 mL by nebulization 4 times daily, Disp: 360 mL, Rfl: 11
- lisinopril (ZESTRIL) 20 MG tablet, Take 1 tablet (20 mg) by mouth daily, Disp: 30 tablet,

Rfl: 2

- metFORMIN (GLUCOPHAGE) 500 MG tablet, Take 1 tablet (500 mg) by mouth 2 times daily, Disp: , Rfl:
- metoprolol succinate (TOPROL XL) 50 MG XL tablet, Take 1 tablet (50 mg) by mouth 2 times daily, Disp: 60 tablet, Rfl: 11
- morphine 10 MG/5ML solution, Take 2.5 mL (5 mg) by mouth every 6 (six) hours if needed for Pain, Disp: 50 mL, Rfl: 0
- omeprazole (PRILOSEC) 40 MG capsule, Take 1 capsule (40 mg) by mouth 1 (one) time each day Indications: Gastroesophageal Reflux Disease, Disp: 90 capsule, Rfl: 1
- predniSONE 20 MG Oral Tablet, Days 1-3 take 1.5 tablets daily Days 4- take 1 tablet daily thereafter until instructed by pulmonary, Disp: 32 tablet, Rfl: 0
- semaglutide (OZEMPIC, 0.25 OR 0.5 MG/DOSE,) 2 MG/3ML injection, Inject 0.25 mg into the skin once a week Indications: Type 2 Diabetes, Disp: 3 mL, Rfl: 0
- sulfamethoxazole-trimethoprim (BACTRIM DS) 800-160 MG per tablet, Take 1 tablet by mouth three times a week for 75 doses, Disp: 12 tablet, Rfl: 5
- theophylline (THEO-24) 300 MG SR capsule, Take 1 capsule (300 mg) by mouth daily, Disp: , Rfl:

ALLERGIES

Allergies

Allergen

- Trazodone Hcl
Violent dreams
- Sulfa Antibiotics
- Probiotic
Bloating, gas, abdominal pain, constipation

Reactions

Other/Unknown (See Comments)

Tachycardia

Other/Unknown (See Comments)

PHYSICAL EXAM

VITAL SIGNS:

ED Triage Vitals [06/19/26 2139]

Temp	Temp	Pulse	Resp	BP	SpO2
	src				
98.5	Oral	(!) 118	20	(!) 152/69	97 % --
°F					
(36.9					
°C)					

Visit Vitals

BP	(!) 176/84
Pulse	(!) 118
Temp	98.5 °F (36.9 °C) (Oral)
Resp	16
Ht	5' 4" (1.626 m)
Wt	181 lb (82.1 kg)
LMP	01/04/2012

SpO2	98%
BMI	31.07 kg/m ²
OB Status	Postmenopausal
Smoking Status	Former
BSA	1.93 m ²

Physical Exam

Constitutional:

Appearance: Normal appearance.

Comments: **Intermittently anxious**

Cardiovascular:

Rate and Rhythm: Regular rhythm. **Tachycardia** present.

Pulses: Normal pulses.

Pulmonary:

Effort: Pulmonary effort is normal.

Comments: **Diminished breath sounds bilaterally speaking in full sentences**

Neurological:

Mental Status: She is alert.

EKG

N/a

RADIOLOGY/PROCEDURES

No orders to display

LABS

Recent Results (from the past 24 hours)

CBC W AUTO DIFF

Collection Time: 06/19/26 9:52 PM

Result	Value	Ref Range
White Blood Cell Count	11.5 (H)	4.0 - 10.4 THOUS/uL
Red Blood Cell Count	3.20 (L)	3.90 - 5.0 MIL/uL
Hemoglobin	9.3 (L)	11.6 - 14.9 GM/DL
Hematocrit	31.3 (L)	38.0 - 48.0 %
Mean Corpuscular Volume	97.8	81.0 - 98.0 FL
Mean Corpuscular Hemoglobin	29.1	27.0 - 33.0 PG
Mean Corpuscular Hemoglobin	29.7 (L)	33.0 - 37.0 G/DL

Conc		
Rdwcv	18.4 (H)	11.5 - 14.5 %
Rdwsd	66.0 (H)	35.0 - 42.0 FL
Platelet Count	318	130 - 400 THOUS/uL
Mean Platelet Volume	9.6	7.4 - 10.4 FL
Nucleated Red Blood Cells	0.3 (H)	0.0 /100 WBC'S
Abs NRBC	0.0	0.0 THOUS/uL
Differential Type	MANUAL	
Seg	80.9 (H)	42.2 - 75.2 %
Band	1.7	0.0 - 5.0 %
Lymphs	11.3 (L)	20.5 - 51.1 %
Monocytes	4.3	1.7 - 9.3 %
Metamyelocyte	0.9 (H)	0 %
Myelo	0.9 (H)	0 %
Abs Neut	9.50 (H)	1.7 - 8.3 THOUS/UL
Abs Lymph	1.30	0.8 - 5.3 THOUS/uL
Abs Mono	0.49	0.1 - 1.0 THOUS/uL
Ovalocytes	1+ (A)	NEGATIVE
Tear Drop Cells	1+ (A)	NEGATIVE
Stomatocytes	1+ (A)	NEGATIVE
Basophilic Stippling	PRESENT (A)	NEGATIVE
META ABS	0.10 (H)	0.0 THOUS/uL
MYELOCYTE ABS	0.10 (H)	0 THOUS/uL

COMPREHENSIVE METABOLIC PANEL

Collection Time: 06/19/26 9:52 PM

Result	Value	Ref Range
Glucose	109 (H)	70 - 99 MG/DL
Blood Urea Nitrogen	13	8 - 23 MG/DL
Creatinine	0.7	0.4 - 1.1 MG/DL
Sodium	142	136 - 145 MMOL/L
Potassium	3.7	3.5 - 5.4 MMOL/L
Chloride	98	98 - 107 MMOL/L
Co2	32	20 - 33 MMOL/L
Calcium	10.1	8.7 - 10.4 MG/DL
Protein Total	6.3	6.0 - 8.2 G/DL
Albumin	4.2	3.4 - 4.8 G/DL
A/G Ratio	1.9	0.8 - 2.0
Bilirubin, Total	0.4	0.3 - 1.2 MG/DL
AST(SGOT)	13	0 - 39 U/L
Alkaline Phosphatase	65	25 - 130 U/L
Alt (SGPT)	19	7 - 35 U/L
Est GFR	>90	>90 ML/MIN/1.73sq.m
GFR Comment		

Reported eGFR is based on the CKD-EPI 2021 equation that does not use a race coefficient.

Anion Gap	12	7 - 14 MMOL/L
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BLOOD GAS VENOUS

Collection Time: 06/19/26 9:52 PM

Result	Value	Ref Range
pH Venous	7.40	7.31 - 7.42
pCO2 Venous	62.8 (H)	39 - 55 MMHG
HCO3, Venous	39.5 (H)	22 - 28 MMHG
Co2 Content Venous	41.4 (H)	23 - 29 MMOL/L
Base Excess	12.4 (H)	0

CK (CPK) CREATINE PHOSPHOKINASE

Collection Time: 06/19/26 9:52 PM

Result	Value	Ref Range
Creatine Kinase Total	22 (L)	26 - 140 U/L

FUTURE APPOINTMENTS

Future Appointments

Date	Time	Provider	Department	Center
6/24/2026	10:00 AM	Kang Smith, Jung Wook, MD	DCMY	DC MIDTOWN
6/30/2026	3:30 PM	Kang Smith, Jung Wook, MD	DCMY	DC MIDTOWN
7/9/2026	4:00 PM	McFadden, Danielle R, NP	DPULMCCG	GH
7/17/2026	2:00 PM	Vardi, Dan, MD	CRIT CARE	DH
7/29/2026	2:40 PM	Vardi, Dan, MD	CRIT CARE	DH
8/19/2026	11:30 AM	Kasel, Mary, NP	DCPALL	DC
8/19/2026	1:40 PM	Vardi, Dan, MD	DH COPD	DH

COURSE & MEDICAL DECISION MAKING

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Monocytes	4.3	1.7 - 9.3 %
Metamyelocyte	0.9 (H)	0 %
Myelo	0.9 (H)	0 %
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Tear Drop Cells	1+ (A)	NEGATIVE
Stomatocytes	1+ (A)	NEGATIVE
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MYELOCYTE ABS	0.10 (H)	0 THOUS/uL

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Protein Total	6.3	6.0 - 8.2 G/DL
Albumin	4.2	3.4 - 4.8 G/DL
A/G Ratio	1.9	0.8 - 2.0
Bilirubin, Total	0.4	0.3 - 1.2 MG/DL
AST(SGOT)	13	0 - 39 U/L
Alkaline Phosphatase	65	25 - 130 U/L
Alt (SGPT)	19	7 - 35 U/L
Est GFR	>90	>90 ML/MIN/1.73sq.m
GFR Comment		

Reported eGFR is based on the CKD-EPI 2021 equation that does not use a race coefficient.

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Co2 Content Venous	41.4 (H)	23 - 29 MMOL/L
Base Excess	12.4 (H)	0

CK (CPK) CREATINE PHOSPHOKINASE

Collection Time: 06/19/26 9:52 PM

Result	Value	Ref Range
Creatine Kinase Total	22 (L)	26 - 140 U/L

Old charts were reviewed per EPIC EMR. Pertinent details are summarized above. All laboratory, radiologic, and EKG studies that were performed in the Emergency Department were a necessary part of the evaluation needed to exclude unstable or emergent medical conditions.

Pertinent studies were reviewed as above.

The patient received:

Medications

ipratropium-albuterol (DUO-NEB) 0.5-2.5 (3)

MG/3ML nebulizer solution 3 mL (3 mL

Nebulization Given 6/19/26 2151)

The initial differential diagnosis for this patient's presentation include: Severe end-stage COPD, debility,

Each of the above mentioned unique tests were ordered by myself for workup of this patient.

I have reviewed the external records available to me, pertaining to this patient's care.

I have reviewed the results of each unique tests mentioned above.

The Independent Historian, nursing, paramedics, was used to gather history pertaining to this patient's care.

My Independent Interpretation of this patient's history, exam and workup (including laboratory, radiology or ancillary testing) are most consistent with severe end-stage COPD, debility. Inability to care for self at home and poor social situation. It is not safe to discharge her back to her current living situation. I do feel she needs likely a skilled nursing facility. Will admit to hospitalist for case management consultation and PT consultation likely placement.

The complexity of problems addressed was high

The risk of complications and/or morbidity or mortality of patient management were high, admit to hospital

Patient's prescriptions were reviewed with changes noted below.

FINAL IMPRESSION

	ICD-10- CM	ICD-9- CM
1. COPD, very severe (HCC)	J44.9	496
2. Debility	R53.81	799.3

- acetaminophen (TYLENOL) 500 MG tablet
- albuterol 108 (90 Base) MCG/ACT inhaler
- Albuterol Sulfate (2.5 MG/3ML) 0.083% Inhalation Nebulization Solution
- Alcohol Swabs 70 % PADS
- amoxicillin-clavulanate (AUGMENTIN) 875-125 MG per tablet
- aspirin 81 MG chewable tablet
- atorvaSTATin (LIPITOR) 20 MG tablet
- Blood Glucose Monitoring Suppl w/Device KIT
- budesonide (PULMICORT) 0.5 MG/2ML nebulizer suspension
- busPIRone (BUSPAR) 5 MG tablet
- docusate 100 MG CAPS
- dupilumab (DUPIXENT) 300 MG/2ML pen
- ferrous sulfate 325 (65 Fe) MG tablet
- Fingerstix Lancets MISC
- FLUoxetine (PROZAC) 20 MG capsule
- furosemide (LASIX) 20 MG tablet
- glucose blood VI test strip
- guaiFENesin (MUCINEX) 600 MG SR tablet
- ipratropium-albuterol (DUO-NEB) 0.5-2.5 (3) MG/3ML nebulizer solution
- lisinopril (ZESTRIL) 20 MG tablet
- metFORMIN (GLUCOPHAGE) 500 MG tablet
- metoprolol succinate (TOPROL XL) 50 MG XL tablet
- morphine 10 MG/5ML solution
- omeprazole (PRILOSEC) 40 MG capsule
- predniSONE 20 MG Oral Tablet
- semaglutide (OZEMPIC, 0.25 OR 0.5 MG/DOSE,) 2 MG/3ML injection
- sulfamethoxazole-trimethoprim (BACTRIM DS) 800-160 MG per tablet
- theophylline (THEO-24) 300 MG SR capsule

Last reviewed on 6/19/2026 9:41 PM by Richardson, Emmitt, RN

Note: Some of the medications listed above may have been ordered after the last review.

A handwritten signature in black ink that reads "William Jones". The signature is written in a cursive, flowing style.

William T Jones, MD

Jones, William T, MD
06/19/26 2305